

Sentinel - Pitch and Demo Guide

Current stage, roadmap, support needed, and your three spoken demos

Prepared for Dhansika | Incubation visit | Live app demo

1. Start With the Story

Mental health professionals often work under time pressure while trying to understand complex patient data. Sentinel was created to support - not replace - their clinical decision-making, by bringing the right information together in one place: a patient's physiology, their own words, and the context a clinician needs to act.

Sentinel is designed to support mental health professionals, not replace clinical judgment.

Then describe the system in one breath: a patient wears a smart ring and journals; the platform fuses physiology, language, and mood; a deterministic crisis engine escalates through a staged protocol that any psychologist can halt with one tap; and the whole thing runs offline at zero monthly cost.

2. Current Stage - Where Sentinel Is Today

- Done: Research completed.
- Done: AI architecture designed.
- Done: Working software prototype - 98/98 tests passing, benchmarked end to end.
- Done: Documentation completed (research paper, white paper, validation report).
- Next: Clinical validation (planned).
- Next: Hardware prototype / live ring pilot (planned).
- Next: Pilot deployment (planned).

3. Roadmap - One Page

- Research (complete) - literature, architecture, psychophysiological foundation.
- Prototype (complete) - working dual-portal software with crisis engine and offline AI.
- Clinical validation (next) - a partner institution, a defined cohort, measured outcomes.
- Pilot - one counselling centre or university counselling office running daily.
- Hardware - live BLE/vendor ring ingestion at scale (M0 SDK is already done).
- Deployment - campus or clinic-wide rollout on commodity hardware.
- Scaling - multi-site deployment, managed tier, FHIR-adjacent export for regulated use.

Read it as a staircase: each stage unlocks the next, and the two steps below the current line are already finished and verifiable.

4. Support Needed - How the Incubator Can Help

- Clinical mentorship - guidance from practising psychologists and psychiatrists.
- Validation guidance - designing a sound clinical validation study.
- Product feedback - iterating the interface with real users.
- Industry connections - clinics, universities, and wearable device makers.

- Regulatory guidance - the path to compliance and clearance.
- Grants for hardware pilot testing - funding for more rings and devices so a real pilot can run.

Be specific when asked: the fastest help is a clinical partner for a validation pilot and access to hardware for real-world ring testing.

5. Your Three Demos

5.1 Thirty seconds (elevator)

Sentinel is a mental-health platform that connects patients, psychologists, and trusted contacts in real time. A patient wears a smart ring and journals; the system fuses physiology, language, and mood to flag risk early. When a patient is in crisis, it escalates automatically through a staged protocol that any psychologist can stop with one tap. It runs fully offline on open-source software at zero monthly cost - and it is designed to support clinicians, not replace them. It is already built and tested.

5.2 Two minutes (room pitch)

Mental health professionals are under time pressure and patient risk evolves between sessions. Most tools only see one signal - an app, a wearable, a chat. [Open the app.] This is Sentinel. The patient portal captures biometric trends from a smart ring and AI-summarized journals, with emotion labels - 28 categories - feeding every summary. The psychologist side ranks patients by risk and auto-expands crisis cases. The key piece is the crisis engine: a staged escalation - patient feedback, trusted-contact email with a signed link, then helpline - that a clinician halts with one tap. Everything is explainable, and the AI path runs locally, so no patient data leaves the clinic. It is validated: 98 tests passing, 96 percent discrepancy accuracy at 0.1 milliseconds. Today it is at the clinical-validation stage, and that is exactly where I need an incubator's help.

5.3 Five minutes (full walkthrough)

- 0:00-0:30 Story opener: the sentence from section 1.
- 0:30-1:30 Log in as the patient (alaya): show biometric trends and one AI journal summary with emotion labels; note the warm patient-facing tone.
- 1:30-3:00 Log in as the psychologist (cel): open the triage dashboard, expand a patient to show the five bio metric cards and the clinical insight, then open the 'Why this summary?' explainability panel.
- 3:00-4:00 Trigger a crisis from the patient view; show it appear live on the psychologist dashboard; acknowledge it and point out the frozen escalation timer. Name the staged protocol.
- 4:00-5:00 Close with validation: 98/98 tests, 96% discrepancy accuracy, 0.1 ms rule engine, 1.4-1.7 s local AI, 7-41 ms REST. End on the one-line ask: clinical partner + hardware for a validation pilot.

Full click-through instructions and demo accounts are in 09_Demo_and_Evaluation_Guide. Rehearse each script out loud at least three times - especially the 30-second one.

6. Notes Before Tomorrow

- You are the product as much as the documents are - lead with the story, then the numbers.
- Keep the app warm: start the backend and Ollama, and click through the demo twice before presenting.
- If the live demo fails, fall back to the 2-minute room pitch - it needs no screen.

- You built this alone, as a 10th-grade student, over three years of independent research - say it plainly once; it is a strength, not a caveat.

Companion documents: Executive Summary (01), Validation & Timing Report (04), Demo & Evaluation Guide (09), Team & Credentials (11).